Western Cape Government FOR YOU

New Somerset Hospital
Department of Anaesthesia

PATIENT STICKER
Name .....
Folder no. .....
Date of birth .....

REGIONAL ANAESTHESIA CONSENT — Lower Limb Nerve Block

1. PLANNED PROCEDURE & BLOCK

Procedure ..... Side: Left Right Bilateral

Block(s): ☐ Fascia iliaca ☐ PENG ☐ Femoral ☐ Lat. femoral cutaneous ☐ Adductor canal
☐ IPACK ☐ Obturator ☐ Lumbar plexus (psoas) ☐ Sciatic (proximal) ☐ Popliteal sciatic
☐ Ankle Other .....

Technique: Single injection Catheter With: ☐ Block alone ☐ Sedation ☐ GA ☐ Spinal ☐ Site marked

Patient has capacity to consent: Yes No If no, consent given by (name / relationship) .....

2. BENEFITS DISCUSSED

why a block is recommended for this patient

- ☐ Excellent pain relief at the operation site (often 8–24 h)
- ☐ Less opioid needed: less nausea, vomiting, drowsiness, itch, constipation
- ☐ Hip fracture: analgesia before surgery, easier positioning for spinal, may reduce delirium
- ☐ Motor-sparing blocks preserve strength → earlier walking & physio
- ☐ Avoid GA (e.g. foot/ankle surgery with popliteal / ankle block)
- ☐ Earlier eating, drinking, mobilising and discharge

Other benefit .....

3. RISKS DISCUSSED

most patients have no complications; risks were weighed against the benefits

	General — all nerve and plane blocks	How often
☐	Numbness / heaviness of the blocked area (usually 8–24 h, up to 48 h)	*Expected*
☐	Block fails or works partly: top-up, extra analgesia or GA	**Common** 1 in 10–100 · ≈ 1 in 10–20
☐	Rebound pain as the block wears off: start oral analgesia early	**Very common** > 1 in 10
☐	Bruising or tenderness at the injection site	**Common** 1 in 10–100
☐	Temporary tingling, numbness or weakness after the block should have worn off	**Common** 1 in 10–100 · resolves in days–weeks
☐	Bleeding / haematoma (higher with anticoagulants)	**Uncommon** 1 in 100–1 000
☐	Local anaesthetic toxicity: seizures, arrhythmia (monitored; treatment available)	**Rare** 1 in 1 000–10 000
☐	Nerve damage lasting more than 6 months / permanent	**Rare** 1 in 1 000–10 000 · ≈ 1 in 2 000–5 000
☐	Infection / abscess at the injection site (higher with catheters)	**Very rare** < 1 in 10 000
☐	Severe allergic reaction to local anaesthetic	**Very rare** < 1 in 10 000
	Block-specific	
☐	Quadriceps weakness → knee gives way → falls	**Common** 1 in 10–100 · less with ACB / PENG
☐	Foot drop; pressure injury to numb heel / foot	**Uncommon** 1 in 100–1 000
☐	May mask compartment syndrome (high-risk fractures): monitoring plan agreed with surgeon	**Rare** 1 in 1 000–10 000
☐	Lumbar plexus: epidural spread (hypotension, bilateral block), retroperitoneal / renal haematoma	**Rare** 1 in 1 000–10 000
☐	Catheter: leak, dislodgement, blockage; infection risk rises after 48–72 h	**Common** 1 in 10–100
☐	Other risk discussed	.....

4. ALTERNATIVES DISCUSSED

- ☐ Oral / IV analgesia (incl. opioids or PCA)
- ☐ Local anaesthetic infiltration by surgeon
- ☐ Spinal / epidural
- ☐ General anaesthetic without a block
- ☐ A different block

5. AFTERCARE ADVISED

- ☐ Do not stand or walk unassisted until strength returns
- ☐ Protect the numb area from heat, cold, pressure and injury
- ☐ Regular analgesia before the block wears off
- ☐ Report pain out of proportion / tight swollen limb
- ☐ Report numbness / weakness lasting > 48 h

6. CONFIRMATION

Right to refuse explained (no effect on surgery or care): Yes No ☐ Questions answered

Patient leaflet: QR shown Printed copy Declined

Interpreter: Not needed Used Language ..... Name .....

Patient gives consent for the block(s) above: YES NO

Patient leaflet

Doctor ..... MP no. ..... Signature .....

Date ..... Time .....

Patient leaflet: omicronblade.github.io/Anaes-Doc-Repo/anaesthesia-forms/leaflets/regional-lower-limb.html

SASA/SASRA Guidelines 2016 · McCombe & Bogod, Anaesthesia 2021 · frequencies approximate